So Ms Wayne, as this is your first appointment with me I'd like to ask you a few questions and get to know a little bit about your medical history. It's also a chance for you to ask me any questions you might have.

F *Sure.*

M *I understand from your GP that you have arthritis.*

F *That's right, yes. It's in my hands and wrists mainly. The **(1) finger joints** in my hands get really puffed up when they're swollen like they are now. The pain can get so bad that it stops me doing a lot of very simple things. Things like having a shower or getting dressed. Then there's the trouble I have making breakfast. Anything where I have to **(2) grip** and move my wrist, like holding the kettle and pouring, or buttering some toast. Well, you get the idea. It's not like I'm an old woman – I'm only 40!*

M *I see. Tell me about your work.*

F *I used to be a **(3) hairdresser**, but I had to give it up after my hands started*

getting really bad. I couldn't just sit around the house and do nothing though, so now I go to the local primary school where my kids are and help the children with their reading. And just lately I've started helping out on a telephone helpline for people with disabilities. They have these special hands-free phones, so it means I can do the work without needing to use my hands so much. It's only a volunteer position, but I really feel like I'm doing something worthwhile.

M *Is your mobility affected at all by the arthritis?*

F *I do have trouble getting in and out of bed, or out of a bath. I also need help getting out of a chair if it's a bit low. I move around the house with the walking aid I have, and when I go out I tend to use a* ***(4)wheelchair****. Our* ***(5) car*** *has also been* ***modified*** *so at least I'm independent – I can go to the shops or out with friends.*

M *I'm guessing you've had physio treatment before.*

F *Yes. In the past it's mainly consisted of heat treatment, but now I'm really interested in some sort of* ***(6) gentle exercises*** *if that's at all possible. I also really want to make sure that my* ***(7) hand splints*** *are still helping and not making the condition worse.*

M *Sure, I can look at that for you.*

F *I've also been to an* ***(8) occupational therapist*** *before. I needed help when things started getting really difficult around the house. I found her very helpful – she had lots of gadgets to help me do things like dressing and cooking, and so many good ideas about how to help with my* ***(9) tiredness*** *without making the pain worse.*

M *Okay. We can talk more about some of the techniques you use to help you at our next appointment. What about your medication.*

F *I take analgesics for the pain mainly these days. I used to take those, you know, those* ***(10) anti-inflammatory drugs****, but the benefits seemed to wear-off and they started making me feel sick, so I had to stop taking them. I do sometimes get a* ***(11) steroid injection****, but not very often. It works really well at reducing the pain for a few weeks, but it hurts like hell. Doctor Wong has mentioned a steroid*

*tablet I could take which will work in a similar way, so we might be looking into that soon. And, ah, a couple of times a day I have an **(12) analgesic cream** that I rub into my hands. I don't know if it really works very well though.*

So, Mr Dafnis, this appointment is a chance for us to talk a little about your upcoming surgery, and for you to ask me any questions you might have.

M *Okay, yes, well, I don't really have any questions about the surgery itself – I feel I understand the procedure quite well from all of my reading – but I do have a concern. I'm actually quite worried about how I'll go with my **(1) eczema** when I come into hospital. It's important for me to follow my usual routine you see or it will flare up again.*

F *What is it specifically that you're worried about?*

M *Well, I've had eczema for years now, since I was a kid. It's chronic, I guess you'd say, and there are all sorts of things that make it worse, like getting too **(2) hot**, you know, like if I'm in a room and the sun is shining in through the window and it starts getting very stuffy or something.*

F *Okay, we can make sure that you have a bed well away from any windows if that'll help.*

M *Yes, that would be good. Thank you. As long as I'm in an air-conditioned room and it isn't too hot, that usually helps. I also notice that certain (3**) foods and drinks** make it worse. I stopped drinking alcohol years ago because it always made the itching worse. But I also have a list of foods that seem to aggravate things so I was wondering if it's possible to give that to someone, or if I can let the hospital know so that they don't include any of those foods in my meals.*

F *If you contact the hospital directly, the'll be able to help you with that.*

M *Great.*

F *Tell me Mr Dafnis, what's your skin like at the moment?*

M *Not very good. You can see by my face how red it is, and the **(4) itching and scratching** has been much worse lately, I think because of the **(5) stress** I'm feeling about the operation. It's also really bad on my **(6) back and my legs**. When I lie down in bed at night sometimes I can hardly stand it.*

F *Do you ever get any sores or weeping?*

M *No, my skin is just dry and very itchy, but I'm most worried that because of the operation any broken areas in the skin will mean I'm at **(7) risk of infection**. I'm doing everything I can to prepare properly though – instead of showers I've been taking **(8) lukewarm baths**, and I never use any soap because it leaves my skin extra dry. I know using it removes the natural oils, so I use a **(9) soap substitute** instead. I'm being really careful about the clothes I wear to be sure they aren't going to exacerbate the itching – I really don't want to be scratching it at all if I can help it – and I've been using an **(10) oily moisturiser** nearly every hour. I don't like to use a lot of those steroid creams and things, but I can if you think it'll help. I see a dermatologist sometimes and he said that if I don't like using them, I can save them as a last resort.*

F *Okay, it sounds like you're doing everything you can to manage the condition. But you do need to try and relax and make sure you're getting plenty of rest. I know that stress is only going to make the condition worse.*

M *You're right, I knowI'm probably getting worked up over nothing and everything will be fine. I'm taking an **(11) antihistamine** at the moment, so that's helped with the scratching and I am **(12) sleeping** a bit better as well.*

You hear a doctor talking to a patient who has been having trouble with his water.

The doctor wants to confirm if the patient

Select one:

- A. has had burning when passing water.
- B. has a blockage in their kidneys.
- C. has high blood pressure.

Feedback

Your answer is correct.

Transcript

Doctor *You can get dressed now…Well, Mr. Hartley, I think your problem is due to the fact that the kidneys are not working as well as they should be. Have you ever had trouble with your water before?*

Patient *Yes. A couple of years ago I had burning when I passed water. The GP said I had high blood pressure and he put me on tablets and it cleared up for a while. So what now?*

Doctor *Well, I want you to have some blood tests, X-rays and kidney function tests. I want you to collect your urine for twenty-four hours. Nurse will tell you exactly how to do it. Then we'll ask you to come in for an ultrasound examination of the kidneys. This is a very simple procedure to make sure there is no obstruction.*

The correct answer is: has a blockage in their kidneys.

Information

Flag question

Information text

Question 2 Audio

Question 2

Incorrect

Mark 0.00 out of 1.00

Flag question

Question text

You hear a nurse briefing her colleague about a patient.

What does she want her colleague to do?

Select one:

- A. Check the patient is receiving the correct amount of fluids.
- B. Order another chest radiograph to check for an effusion.
- C. Confirm the patient is stable and check his blood culture.

Feedback

Your answer is incorrect.

Transcript

Nurse *Our sickest patient is AJ. AJ is a 6-year-old boy who was admitted with hypoxia and respiratory distress secondary to a left lower lobe pneumonia. He presented with cough and high fevers for 2 days before admission, and on the day he presented to the emergency department he had worsening respiratory distress. In the emergency department he was found to have an Na of 130, likely secondary to volume depletion versus syndrome of inappropriate secretion of antidiuretic hormone. He received a fluid bolus and was started on O2 at 2.5 L. He's on Ceftriaxone. Please look in on him at approximately midnight and make sure his vitals are unchanged and his oxygen saturation is stable. Also check to determine if his blood culture is positive tonight. If his respiratory status worsens, please get another chest radiograph to determine if he is developing an effusion.*

The correct answer is: Confirm the patient is stable and check his blood culture.

Information

Flag question

Information text

Question 3 Audio

Question 3
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

You hear part of a consultation at a GP practice with a patient suffering from depression and sleep disturbances.

The GP recommends cognitive behavioural therapy because

Select one:

- A. there are better ways for the patient to respond to life's challenges.
- B. it will help the patient better manage the way he feels when stressed.
- C. the patient needs to control his anger and medication isn't working.

Feedback

Your answer is incorrect.

Transcript

GP	*Well Brian, at the Belmont Private Hospital they have a CBT programme, which stands for cognitive behavioural therapy. It focuses on changing your thinking patterns and the way you respond to challenging situations.*
Brian	*I see.*
GP	*It will give you the skills and knowledge to help you achieve more control of your emotions and to feel better about yourself and your life.*
Brian	*That sounds like it could be a good thing for me.*
GP	*Other topics CBT deals with include anger management, coping with change, as well as things like setting goals and assertiveness training.*
Brian	*Sounds okay.*
GP	*All right, Brian, I'll book you in. It's a 3-week course every day from 9am until 3pm. Now, I would like you to take the antidepressant medication one at night an hour before you go to bed. You can still take the normison if you are having trouble sleeping.*

The correct answer is: it will help the patient better manage the way he feels when stressed.

Information

Flag question

Information text

Question 4 Audio

Question 4

Correct

Mark 1.00 out of 1.00

Flag question

Question text

You hear a nurse seeking advice from a doctor.

Why does the nurse telephone the doctor?

Select one:

- A. She would like to know how to proceed with the patient.
- B. The patient is in intense pain and has a fever.
- C. She suspects that the patient has an infection.

Feedback

Your answer is correct.

Transcript

Nurse *Hi Doctor Jones, this is Helen from the PA Hospital. I'm calling about your patient, Mrs Singh, in room 231. She's a 70-year-old, second day post-op hip patient and she's been complaining of intense pain tonight. She received all her scheduled antibiotics but started running a fever of 38.3 degrees at midnight. Her incision is also quite red and I noted some new purulent drainage. I'm concerned she may have an infection and I'd like you to see Mrs Singh as soon as possible, but in the meantime, I'm wondering what course of action you'd like me to take. Whether or not you'd like me to draw a CBC or blood cultures?*

The correct answer is: She would like to know how to proceed with the patient.

Information

Flag question

Information text

Question 5 Audio

Question 5
Correct
Mark 1.00 out of 1.00

Flag question

Question text

You hear the principal of an aged care home talking to staff about osteoporosis.

She wants to highlight that

Select one:

- A. there are now treatments available for men.
- B. residents are not getting enough calcium and vitamin D.
- C. a lot of the residents may already have osteoporosis.

Feedback

Your answer is correct.

Transcript

Principal

I wanted to finish up by talking to you about osteoporosis. The age of a lot of our residents means that they are at risk of developing the disease. Genetics and lifestyle factors play a part, but not getting enough calcium and vitamin D can make the condition worse, so I'd like you to ensure that the needs of our residents are being met in this regard. Also, and this may be a significant factor for some of our residents, corticosteroids may contribute to the onset of the disease. For residents who already have osteoporosis, there are medications available to help, including Fosamax, Boniva, Actonel and Reclast. These drugs work by increasing bone density, so they can help reduce the risk of fractures. Now, there are also treatments available specifically for men, which wasn't the case in the past, and as you know some of our male residents can be resistant to taking medication, so I'd really like you to mention this to them.

The correct answer is: there are now treatments available for men.

Information

Flag question

Information text

Question 6 Audio

Question 6

Correct

Mark 1.00 out of 1.00

Flag question

Question text

You hear two nurses discussing a patient, Mrs Robbins.

Julie says Mrs Robbins refused to mobilise because

Select one:

- A. she was happy having a sponge bath.
- B. she wanted to hear directly from the doctor.
- C. she was upset and wanted to be left alone.

Feedback

Your answer is correct.

Transcript

Nurse 1 *Can I talk to you for a minute about Mrs Robbins in room 225?*

Nurse 2 *Sure. What's happening?*

Nurse 1 *Well, as you know, she's Doctor Liu's patient, and his notes state that she needs to start mobilising today and that her discharge is planned for the day after tomorrow. So, I checked in on her and asked how she was feeling and let her know that I'd take her for a shower. She's been getting a sponge bath up till now.*

Nurse 2 *Okay.*

Nurse 1 *The thing is, the moment I mentioned the shower to her she started getting really upset and said that she just wanted to rest. I tried explaining how important it was for her to mobilise. How it was what the doctor had ordered and that it would help speed-up her recovery, but she wasn't hasn't any of it. She told me she'd speak to Doctor Liu herself, and that she wouldn't listen to me because I was only the nurse. I didn't want to make her any more upset than she already was, so I just left it, but now I don't know what to do.*

The correct answer is: she wanted to hear directly from the doctor.

Why does Dr Munro say he volunteered to treat Ebola patients in Liberia?

Select one:

- A. because of the high death rate from Ebola in Africa.
- B. because medical treatment in Africa can be aggressive.
- C. because he believed he had a duty to try and help.

Feedback

Your answer is correct.

Transcript

Interviewer *Today we're speaking to Dr Kyle Munro, a critical care physician from the United States who's recently returned from Liberia where he volunteered to treat Ebola patients. Can you tell us a little about Ebola, and explain your reasons behind volunteering?*

***Question 1**

Dr Kyle Munro *Sure. Ebola virus is a severe, often fatal illness and without treatment nine out of 10 people who contract the virus die, many from dehydration, hyperbulemia from excessive vomiting, loss of nutrients due to diarrhea, internal and external bleeding, and sometimes kidney and liver failure. Aggressive medical care is really simple by modern medical standards. Saving lives*

might require as little as clean IV needles, fluids and basic lab tests – things that are readily available in resource-rich parts of the world – and we have them at our disposal all the time, they're not considered fancy where we come from. That isn't always the case in remote parts of Africa, where supplies can be scarce.

The correct answer is: because he believed he had a duty to try and help.

Question 2
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

Dr Munro says the patient's he treated were often afraid because

Select one:

A. there was a high death rate for patients who received treatment.

B. family members didn't believe that treatment was helping.

C. they didn't trust the doctors because they were foreigners.

Feedback

Your answer is incorrect.

Transcript

Interviewer *And how did the patients you were treating respond to your presence?*

***Question 2**

Dr Kyle Munro *To give you some sort of idea of what we were dealing with, the first two patients I treated were a brother and sister. Both had developed Ebola symptoms, which often presents like flu, with fever and pain, and straight away they fled to the bush because they were fearful of what might happen to them if they were treated. They were found quite quickly because they were too weak to run, but even with treatment, both later died. Nine out of ten people we treated, the loved ones of a family who went into a treatment facility, they never saw them again. So the distrust about what we were doing wasn't entirely unfounded.*

The correct answer is: there was a high death rate for patients who received treatment.

Question 3
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

What does Dr Munro say about the specialised clothing he had to wear?

Select one:

A. That he spent more time rehydrating than treating patients.

B. That patients found it similar to what is worn in a hospital.

C. That it affected the way he would treat his patients.

Feedback

Your answer is incorrect.

Transcript

Interviewer *And is it true you had to wear a lot of specialised clothing while treating patients with Ebola?*

Dr Kyle Munro

***Question 3**

Yes, that's correct. It's a little beyond the traditional white coat a lot of patient's associate with doctors in a hospital setting. I would wear my scrubs and then I would put on a pair of thick rubber boots that came up to my knees. After that, I had to get dressed in a body suit, then two pairs of gloves, a face mask, a hood that covered my neck and, finally, goggles.The overall impression is similar to that of a spacesuit. In the tropical heat and humidity, it was suffocating. You lose about three to five litres of sweat, then spend the next two hours hydrating before you go back in. It does limit the level of care you're able to provide because it prevents you from physically contacting your patients, and that reduces the amount of sensory input we usually get as part of our jobs as physicians. But it also saves your life.

The correct answer is: That it affected the way he would treat his patients.

Question 4

Correct

Mark 1.00 out of 1.00

Flag question

Question text

Why does Dr Munro believe the case of the young girl is tragic?

Select one:

A. It highlights how even young children can get Ebola.

B. It demonstrates how families were affected by the disease.

C. It is a situation that only affected a small group of patients.

Feedback

Your answer is correct.

Transcript

Interviewer *Can you tell us about one of the patients you treated while you were there?*

***Question 4**

Dr Kyle Munro *One particular case involved a young girl who was part of a small cohort of patients we had to deal with where someone in the family would be positive and someone else would be negative. And once a person was negative, they had to leave the unit. The little girl was 6-years-old and she had contracted the virus, but her mother had not, so I had no option but to escort her away. Both the child and her mother were terrified about what lay ahead for them. The girl died four days after they were separated. I think, in some ways, this example encapsulates just how awful things could be during this outbreak, and the kind of personal tragedies that people had to live through.*

The correct answer is: It demonstrates how families were affected by the disease.

Question 5

Correct

Mark 1.00 out of 1.00

Flag question

Question text

Treating Ebola patients in Liberia has made Dr Munro

Select one:

A. improve how he manages the emotional needs of his patients.

B. realise that seeing patients suffering is part of a doctor's job.

C. understand that doctors can't do everything for their patients.

Feedback

Your answer is correct.

Transcript

Interviewer *You're back home now in Massachusetts. When treating patients in the US, do you think you approach your job in a different way because of your work in Liberia?*

***Question 5**

Dr Kyle Munro *I do think it's made me sensitive to the limitations that are imposed on us. Making sure that you've made a connection with your patient. One of the things we had to do for our patients in Liberia, in addition to just witnessing their illness and doing what doctors do around the world, which is try to use medicines to blunt the worst effects of an illness, is also to try and help manage their own anxieties and fears. I would like to think that I had been aware of it before, but now I think I understand the principle in an entirely new way.*

The correct answer is: improve how he manages the emotional needs of his patients.

Question 6

Incorrect

Mark 0.00 out of 1.00

Flag question

Question text

Dr Munro believes Ebola may spread to other parts of the world because

Select one:

- A. it takes longer for the disease to develop than people think.
- B. plane travel has made the world a much smaller place.
- C. an infected person in a crows could spread the disease.

Feedback

Your answer is incorrect.

Transcript

Interviewer *And do you believe Ebola is something for the rest of the world to be concerned about?*

***Question 6**

Dr Kyle Munro

Well, I know the disease has spread further, and I'm not surprised. In a recent case in Nigeria, a man infected with Ebola collapsed at a crowded airport in Lagos, its busiest city. He later died and now authorities are closely monitoring up to 59 people he might have come in contact with. The majority of people think that Ebola is a dramatic disease that kills people in no time. The reality is that the incubation period is 21 days, and because the disease can spread from an infected person to another person via direct contact with blood or body fluidsvia the nose, mouth, eyes, or even from open wounds like cuts and abrasions, the potential for spread outside of Africa is there.

The correct answer is: it takes longer for the disease to develop than people think.

Dr Tredellow believes snoring poses concerns because

Select one:

- A. it is possible for anyone to become a snorer.
- B. it contributes to several other health issues.
- C. everyone has an upper airway that collapses.

Feedback

Your answer is correct.

Transcript

***Question 1**

James Tredellow

Hello. I'm James Tredellow, a sleep physician who specialises in upper airway physiology. While snoring can be frustrating for those who have to listen to it, it is also known to contribute to a range of health problems. There's now also an emerging line of research that suggests snoring may directly contribute to serious cardiovascular health problems. When we go to sleep, the muscles of the upper airway relax, causing it to become "floppy" and partially collapse. This occurs to some extent in all of us. However, in some people the airway is excessively narrowed, particularly at the level of the tongue and uvula, or soft palate. As we breathe in, we produce suction pressure to draw air into the lungs. This further narrows the airway – similar to when a person sucks too hard on a straw – and can cause upper airway tissues such as the soft palate to vibrate or flutter, similar to a flag in the wind. This movement creates the noise of snoring.

The correct answer is: it contributes to several other health issues.

Question 2
Correct
Mark 1.00 out of 1.00

Flag question

Question text

Dr Tredellow says the main cause of snoring is

Select one:

- A. drinking too much.
- B. being overweight.
- C. a narrow airway.

Feedback

Your answer is correct.

Transcript

James Tredellow

***Question 2**

There are many triggers for snoring. If a person doesn't normally snore, alcohol can be a key trigger as it blocks the nose and can contribute to relaxing of the airway muscles. And if a person already snores, alcohol is likely to make their snoring a lot louder. Being overweight increases fat around the neck, compressing and narrowing the throat. But thin people do snore too, and many who are overweight do not. A blocked nose – due to a cold, allergies, polyps or anatomical abnormality – creates the need for greater suction pressure to draw air into the lungs when breathing, which further narrows the airway.Mouth opening often occurs when the nose is blocked during sleep, which itself can cause snoring, via airway anatomy and pressure changes. Sleeping on the back contributes to collapse of the airway, as gravity pushes the tongue and soft palate towards the back of the throat. Enlarged tonsils narrow the airway, and are a primary cause of snoring in children.

The correct answer is: a narrow airway.

Question 3
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

According to Dr Tredellow there may be societal impacts from snoring for a person who

Select one:

A. is suffering from obesity.

B. has concentration issues.

C. is worried about their snoring.

Feedback

Your answer is incorrect.

Transcript

***Question 3**

James Tredellow

In terms of the effects that snoring can have on a person's well-being, it can create a dry mouth, sore throat or headache, and can leave a person feeling tired. It can place significant strains on personal relationships, particularly a person's bed partner, but also those sleeping in an adjacent room or even sometimes on the other side of the house! Snoring forces many couples to sleep in separate bedrooms, disrupting intimate bonds. Snorers are often embarrassed to sleep among others, promoting anxiety and hindering their social relationships. Sleep disruption caused by snoring can make someone, and others, cranky and irritable during the day, but also may lead to impaired memory, concentration and learning difficulties, and obesity and reduced immunity.

The correct answer is: is worried about their snoring.

Question 4

Incorrect

Mark 0.00 out of 1.00

Flag question

Question text

What point does Dr Tredellow make about obstructive sleep apnoea?

Select one:

A. snoring sometimes causes people to stop breathing.

B. snoring can be linked to obstructive sleep apnoea.

C. obstructive sleep apnoea and snoring are similar.

Feedback

Your answer is incorrect.

Transcript

***Question 4**

James Tredellow *More serious consequences include snoring as a possible marker for obstructive sleep apnoea, a disorder where the upper airway repeatedly closes during sleep, and breathing stops for at least ten seconds at a time. Obstructions to breathing can sometimes last for more than a minute and occur over 100 times an hour, starving the body of oxygen and fragmenting sleep. Obstructive sleep apnoea sufferers are often excessively sleepy and at greater risk of car and industrial accidents, and cardiovascular diseases like high blood pressure, heart attack and stroke.*

The correct answer is: snoring can be linked to obstructive sleep apnoea.

Question 5

Incorrect

Mark 0.00 out of 1.00

Flag question

Question text

Dr Tredellow feels research on carotoid artery atherosclerosis

Select one:

A. is still in its infancy despite highlighting some serious issues.

B. shows that snoring may be far more dangerous than once believed.

C. demonstrates a clear connection between snoring and heart disease.

Feedback

Your answer is incorrect.

Transcript

James Tredellow

***Question 5**

Over the last decade, research has suggested heavy snoring itself may be a direct cause of cardiovascular complications, particularly a condition known as carotid artery atherosclerosis. When affected by atherosclerosis, fatty deposits known as plaques form in the wall of the artery. Over time these can cause the blood vessel to narrow and limit blood flow to the brain. One study has shown that snoring vibrations are transmitted to the carotid artery, which can damage its wall and lead to the development of atherosclerosis. Subsequently, snoring may rupture a formed plaque, resulting in pieces of the plaque moving through the bloodstream and blocking small vessels in the brain. However, additional research is required to bring further clarity to this hypothesis.

The correct answer is: is still in its infancy despite highlighting some serious issues.

Question 6
Incorrect
Mark 0.00 out of 1.00

Flag question

Question text

What does Dr Tredellow say about the treatment for snoring?

Select one:

- A. Making specific lifestyle changes contributes to a reduction in snoring.
- B. There are too many causes of snoring to be able to treat them all.
- C. Treatment is best provided by a recognised sleep physician.

Feedback

Your answer is incorrect.

Transcript

***Question 6**

James Tredellow

Given the multiple causes of snoring, there are numerous potential treatments available that work for some people but not others. Recommended lifestyle changes that work for some patients may include: avoiding alcohol before bed and avoidingsleep deprivation; losing excess weight; and, stopping smoking. There are several medical treatments for snoring that require professional advice. If you treat patients that snore regularly, it is highly recommended they see a sleep and respiratory doctor for diagnosis, and to determine the right kind of treatment.

The correct answer is: Treatment is best provided by a recognised sleep physician.